

Client #5, here's your *90-day plan*, built around your own goal.

*Metabolically young, cardiovascularly exposed · Iron-depleted ·
Atopic · Under-recovered*

BIOLOGICAL AGE

21.1

3.9 yrs younger than calendar

THREE PILLARS

3

lipids · iron · recovery

HABITS, TOTAL

6

phased over 12 weeks

PHYSICIAN ITEMS

3

lipids · iron · ENT

"Better cardio, less fatigue and higher sharpness."

That's what you told us you want. Here's the good news: your number-one goal, **better cardio**, is also the single biggest lever on your two real blood findings, your lipids and your early insulin signal. The medicine and the goal are the same thing. **Less fatigue** points

straight at your iron and your sleep. **Sharper mind** follows both. We didn't design a plan and bolt your goals on, your goals *are* the plan.

WHAT YOUR ANSWERS CHANGED

Your bloodwork told us what. Your answers told us why.

Part 1 flagged three things from blood alone and marked the causes "to be confirmed." Your questionnaire confirmed or revised every one of them. The headlines:

- **Your family history is reassuring.** The heart issues in your family appeared at 75 to 86, no early heart attacks or strokes. Your inherited Lp(a) is real, but it is *not* amplified by a pattern of premature heart disease in the family. That meaningfully softens the worry, it does not remove the work.
- **Your iron puzzle got clearer, and more interesting.** You're a heavy meat-eater, so "not enough iron" was never the full story. But you eat *zero red meat*, take coffee right after lunch, have bleeding gums and blood in your nasal mucus, train hard every single day, and have travelled rural Bihar and Thailand without ever testing a stool. That's a real list. The cause is almost certainly more than one thing, and it needs a doctor's eye before any high-dose iron.
- **A new theme appeared: you are under-recovered.** Daily MMA, strength and HIIT, on roughly six hours of sleep, with a stress score that sits above 80 every day, and your own words: "feel under-recovered," "perpetually tired." This sits underneath your fatigue, your afternoon crash, and part of your early insulin signal. It is the most fixable thing in this entire report.
- **Your atopy is confirmed.** The high IgE maps cleanly onto a perpetually blocked nose, pollen allergy, childhood asthma and a shellfish anaphylaxis. Not incidental, a real allergic constitution worth managing.

And one thing we will **not** touch: your diet discipline. Zero fried food, no sugar, no sugary drinks, no late-night eating, almost nothing processed. That is top-decile. Piling restriction on top of it would be the wrong move. We make two surgical changes, not an overhaul.

THE STRATEGY

Three pillars, not a checklist of fifteen fixes

Attack the roots and the leaves resolve. Everything in your plan ladders up to one of these three.

PILLAR 1 · CARDIOVASCULAR

Lower ApoB, lift HDL, to offset the inherited Lp(a)

You can't change Lp(a), it's genetic and set for life. What you *can* move is ApoB, the particle count that actually does the damage. Get it down meaningfully and you offset the inherited risk. Your diet is already clean, so the levers that remain are the ones you're not yet pulling.

The moves: Zone-2 cardio (your own goal) · soluble fibre to 30-40g via oats, chia, psyllium and dal, not vegetables you won't eat · swap saturated fat for unsaturated (less ghee, fewer yolks, lower-fat milk) · algal omega-3 · cut the occasional smoking/vaping entirely · physician chat on early ApoB-lowering.

Target by Day 90: ApoB 96 → under 85 · LDL 135 → under 115 · HDL 43 → 48+ · non-HDL 152 → under 135

PILLAR 2 · IRON

Refill the stores, and find the leak

Ferritin 7.5 in a man who eats meat daily is a puzzle worth solving properly, not papering over with a supplement. The plan has two arms: a diet/absorption arm you start now, and a physician arm that finds the cause before any therapeutic iron.

The moves: reintroduce red meat (mutton) 1-2x/week for heme iron · vitamin C with iron meals · move coffee and your calcium supplement away from those meals · **physician:** iron workup + stool occult-blood + parasite test before high-dose iron · ENT for the nasal bleeding source.

Target by Day 90: ferritin 7.5 → 30+ · transferrin saturation 11.5% → 25%+ · the "perpetually tired" and cold hands easing

PILLAR 3 · RECOVERY

Recover as hard as you train

You are not under-trained, you are under-recovered. Training every day on six hours of sleep with no rest day is leaving performance, and biology, on the table. This pillar costs nothing and pays back the fastest.

The moves: sleep 6h → 7-7.5h (bed by 9:45) · one true rest day a week · deload every 4th week · 10-min daily breathwork for the 80+ stress score · convert two weekly HIIT slots into Zone-2 (also serves the cardio goal).

Target by Day 90: HOMA-IR 1.25 → under 1.0 · afternoon crash softened · genuine "less fatigue"

Move the way your goal needs

Right now you do roughly **20 minutes of steady cardio a week** while training hard every day. That single gap is why "better cardio" is your goal and why your HDL is low-ish. Fixing it serves your goal, your lipids and your insulin signal at once. The plan is to *redistribute* your load, not add to it.

WEEKLY STRUCTURE

- **Zone-2 cardio: 150 min/week**, in 3-4 sessions of 35-45 min at a conversational pace (heart rate roughly 120-140). Cycle, incline treadmill, or aerobic shadow-boxing rounds. *During Noida's high-AQI months (Oct-Feb), keep these indoors with filtered air, it matters for your nose and your heart.*
- **HIIT: 1 session/week** (your Saturday Monster-Zero slot). Down from near-daily, the extra intensity is feeding under-recovery and possibly worsening iron loss through training.
- **Resistance: 4 sessions/week.** Keep your trainer-guided push/pull compound work, it's clearly working. Protect the knees: no explosive or plyometric knee loading.
- **MMA: 3-4 sessions/week** (down from daily).
- **One full rest or active-recovery day.** Walk and mobility only. Non-negotiable, this is the recovery pillar in action.
- **Daily 10-min mobility**, knee-led.

THE 12-WEEK PROGRESSION

- **Weeks 1-2, redistribute:** add 2 Zone-2 sessions, claim Sunday as full rest. Add no intensity.
- **Weeks 3-6, build the base:** 3 Zone-2/week, trim HIIT to one. Watch your resting heart rate start to drop and "cardio" start to feel different.
- **Weeks 7-10, progress + first deload:** 4 Zone-2 on lighter weeks. Week 8 is a deload (volume down ~40%). Overload resumes week 9.
- **Weeks 11-13, consolidate:** hold the structure. No heavy training in the 48 hours before your Day-90 retest, hard sessions can skew the blood panel.

Evidence: Zone-2 for metabolic and lipid health [E1]; resistance training maintenance [E1]; deload for recovery [E2].

The cheapest win in this whole report

You sleep about six hours, wake "always groggy," snore on your back, and feel perpetually tired while training daily. Short sleep alone lifts insulin resistance, blunts recovery and flattens mood, it sits under three of your findings. Moving bedtime from 10:30 to **9:45** buys you 45-60 minutes for free, while protecting your 5:45 wake.

- **Bed by 9:45, screens off by 9:15.** You sometimes use screens in bed, this is the one to break. Magnesium glycinate (you already take it) at 9:00. Your room is already perfect: dark, 19°C.
- **Side-sleep, and see an ENT.** You snore on your back and one side of your nose is always blocked. That nasal obstruction is very likely degrading your sleep quality, the real reason behind "always groggy." The ENT referral below ties your nose, your iron and your sleep together.
- **10 minutes of daylight after waking.** You get only 2-3 hours of daylight a day, mostly indoors. Morning light anchors your rhythm and softens both the groggy mornings and the afternoon crash.
- **No caffeine after 1pm.** Specifically drop the after-lunch coffee, it protects your already-short sleep *and* your iron absorption in one move.
- **Beat the afternoon crash with a 10-min walk after lunch,** not a recline. It blunts the post-rice glucose dip that's half the reason you crash.

Evidence: sleep extension for insulin sensitivity and recovery [E1]; morning light for circadian alignment [E2].

Two surgical changes, not an overhaul

Your diet is genuinely excellent and we're protecting it. We change exactly two things: **add soluble fibre without using vegetables you won't eat**, and **shift fat quality while adding heme iron**. Everything is built to survive travel, your stated biggest obstacle.

DAILY TARGETS

- Protein ~140g (you're already there) • fibre 35g, of which 8-10g soluble • keep the Tue/Thu fasts, they're a good habit.

FIBRE, THE NO-VEGETABLE WAY

- **Psyllium husk (isabgol), 1 tsp to 1 tbsp in water daily.** The single highest-leverage move for ApoB and LDL, and it tastes of nothing.
- **Oats (already daily) + 1 tbsp chia + 1 tbsp ground flax** stirred in, soluble fibre plus plant omega-3.
- **Dal up from 2x to 4-5x/week**, plant iron too, paired with vitamin C and kept away from coffee.
- Berries (you like blueberry), guava, apple-with-skin, easy fibre with zero vegetables.

FAT QUALITY + IRON

- Cut the leftover ghee, use groundnut/mustard/olive oil with a light hand.
- On heavy-egg days, go 2 whole + 3 whites instead of 5 whole, same protein, less saturated load. Whites are unlimited.
- Switch coffee/shake dairy to lower-fat lactose-free milk.
- **Reintroduce red meat (mutton) 1-2x/week**, your best absorbable iron, you simply weren't eating it.
- Vitamin C 500mg (you already own it) *with* the iron meal; move coffee and the calcium supplement 1-2 hours away.

A SAMPLE WEEK (USING ONLY FOODS YOU ALREADY EAT)

Mon • train Whey isolate + 2-yolk egg bhurji AM • chicken, kala namak rice, dal + lemon • oats-chia bowl PM

Tue • fast Black coffee only → 2pm: mutton, rice, dal + lemon + vitamin C • chole + rice dinner + psyllium

Wed · train	Oats + chia + flax + whey · chicken + brown rice + dal · egg-white bhurji + multigrain dinner
Thu · fast	Black coffee only → 2pm: chicken, rice, dal + lemon · mutton or chicken + roti dinner
Fri · train	Whey + eggs · chicken + rice + dal + lemon · chole + rice + psyllium
Sat · HIIT	Whey + oats-chia (Monster Zero pre-HIIT fine) · mutton/chicken + rice + dal · chicken + multigrain dinner
Sun · rest	Big egg-white + 2-yolk bhurji + oats · chicken + rice + dal + lemon · light chole + multigrain + psyllium

Lactose rule: isolate protein only, never concentrate (you've felt the difference); lactose-free milk only. Evidence: soluble fibre for LDL/ApoB [E1]; heme iron + vitamin C co-timing [E1]; saturated-to-unsaturated fat swap [E1].

Prune and re-time, don't pile on

Your stack is large and has real redundancy, your B12 over 2000 and folate over 24 come from **three overlapping sources**. The job here is mostly to tidy and re-time, not to add. Net cost likely drops.

Vitamin D3	60k/week is a loading dose and you're already at 63 ng/mL (top of optimal). Step down to 60k/month or 2,000 IU/day.	CHANGE
B12 / folate trio (Recalmin-XT, NVMe, multivit)	Three sources stacking. Consolidate to one (keep the multivitamin). Note: Recalmin also has calcium, don't co-time with iron meals.	CONSOLIDATE
Omega-3	Switch fish oil → algal omega-3, 1.5g+ EPA+DHA/day . Removes any shellfish-allergy doubt and doses up for your lipids.	SWITCH
Vitamin C 500mg	Re-time: take with iron meals (mutton/dal), not as a standalone morning dose.	RE-TIME
Magnesium glycinate	Keep, take in the evening for sleep (serum Mg 1.98, just under optimal).	KEEP
Creatine	Keep, supports training and the "sharpness" goal.	KEEP
Whey isolate	Keep, correct form for your lactose intolerance. Never switch to concentrate.	KEEP
Psyllium husk	Add 1 tsp-1 tbsp/day, highest-leverage veg-free fibre for ApoB.	ADD
Iron	Do not self-start. Physician-gated until the cause workup is done, masking a bleeding source is dangerous.	PHYSICIAN

No NSAID-based recovery aids, ever (your Combiflam/ibuprofen allergy). For soreness: ice, load management, and your physician-approved Zerodol SP sparingly.

Down-regulate a genuinely high stress load

Your stress score sits above 80 every day, and you named the new sources honestly: a relationship you can't wind down from the way you used to, and money pressure as a business owner. You meditate, but do no breathwork, and that's the fastest lever on that number.

- **10 minutes of breathwork daily** (physiological sigh, or 4-7-8). The quickest down-regulator for an 80+ stress score.
- **Make journaling nightly** (you do it inconsistently), useful given the low mood of the last few months.
- The relationship and money stress are real and not a supplement problem. The levers are the breathwork, the protected sleep, and the one rest day. **If the low mood persists past a few more weeks, a check-in with a counsellor is a reasonable, sensible move**, not a sign of weakness.

This is a supporting domain, paired with Recovery. Evidence: breathwork for acute stress and HRV [E2]; expressive writing for mood [E3].

THE 12-WEEK PLAN

Six habits, phased so they actually stick

More than six new habits at once and adherence collapses, even for someone as disciplined as you. We add them in pairs, with a coach check-in at each step.

Weeks 1-2 Foundation

1. Bed by 9:45, screens off 9:15 (protect the 5:45 wake → 7h)
2. Claim Sunday as a full rest day

Day-14 check-in: *Is the earlier bedtime holding, or is work pushing it back? Did the rest day actually happen, or did MMA sneak in? How are the mornings vs baseline?*

Weeks 3-6 Build

3. 3x Zone-2 cardio/week (your "better cardio" goal)
4. Psyllium daily + vitamin C with iron meals + drop the after-lunch coffee

Day-28 + 42 check-in: *Zone-2 adherence, does cardio feel better yet? Is HIIT actually trimmed to one a week? Psyllium tolerance. And critically: are the physician (iron + lipid) and ENT appointments booked?*

Weeks 7-10 Deepen

5. Red meat 1-2x/week + halve egg-yolk volume + lower-fat lactose-free milk
6. 10-min daily breathwork

Day-56 + 70 check-in: *Is mutton back in rotation? Fat swaps sticking? Did breathwork move the stress score? Energy and afternoon-crash trend. Was the week-8 deload respected?*

Weeks 11-13 Retest prep

- Maintain all six. Pre-retest: no heavy training 48h prior, fast 12h, no caffeine 24h, no alcohol 72h, **same lab, same morning slot as Day 1.**

Day-84 + 90 review: *Confirm retest logistics and the physician/ENT outcomes, did we find the iron cause? Lock the same-lab, same-slot retest so the deltas are real, not noise.*

TAKE THESE THREE TO A PHYSICIAN FIRST

Three conversations to have before, or alongside, the protocol

PRIMARY CARE → CARDIOLOGY / LIPIDOLOGY

Lp(a) 49 with ApoB 96 / LDL 135 / non-HDL 152. Confirm Lp(a) once (it's stable for life). Your family history is reassuring, share it. Ask whether early ApoB-lowering is worth it at 25 given the inherited Lp(a). The protocol is already targeting ApoB hard regardless.

PRIMARY CARE + STOOL TESTS

Ferritin 7.5 with bleeding signals, in a heavy meat-eater. This needs a cause, not just a pill. Ask for a stool occult-blood and a parasite/ova test (rural Bihar + Thailand travel, never tested). Let the physician choose and supervise iron repletion, then retest ferritin at 8-12 weeks. **Do not start high-dose iron on your own.**

ENT

One-sided nose always blocked + near-daily Otrivin + nasal bleeding + snoring + always groggy. One referral that ties three threads together: your atopy, a likely iron-loss source, and your poor sleep quality. Ask specifically about a deviated septum and about rebound congestion from chronic Otrivin use.

HOW WE'LL MEASURE IT

The retest plan, so change is measured not assumed

- **Day 30:** subjective, fatigue, morning grogginess and the Bevel stress score should already be moving as sleep extends and the rest day lands.
- **Day 60:** a lipid recheck (ApoB, LDL, HDL, non-HDL), lipids turn over in about six weeks, plus iron studies if the physician has started repletion.
- **Day 90, full panel** (same lab, same morning slot, 12h fasted). Priority remeasures: ferritin, transferrin saturation, ApoB, LDL, HDL, HOMA-IR, hsCRP, HbA1c. Add-ons worth doing: a one-time Lp(a) confirm, stool occult-blood, and a reticulocyte-Hb or soluble transferrin receptor to confirm the iron response.
- **Week 12:** a Vitamin D recheck to confirm the stepped-down dose holds you at 40-60.

One honest expectation to set: your Lp(a) will not change, it's genetic. The win there is offsetting it through ApoB, not lowering the Lp(a) number itself. Everything else on this list should move.

PROTECT WHAT'S WORKING

Three things we're deliberately not touching

1. **Your diet discipline.** Zero fried, no sugar, no late eating, top-decile. We add fibre and shift fat quality, we do not restrict.
2. **Your strength training and MMA progress.** Testosterone 801, good gains, it's working. We redistribute load and add a rest day, nothing more.
3. **Creatine, isolate whey, magnesium glycinate, lactose-free milk.** All already correct. Keep them exactly as they are.

You came in disciplined and motivated, that's the rarest ingredient and you already have it. This plan just points it at the three things your blood and your answers agree on. Better cardio, less fatigue, sharper mind, by Day 90, measured not assumed.

IMPORTANT NOTES

This protocol is generated from a single blood test and your lifestyle questionnaire. It is **not a medical diagnosis or a prescription**. Every supplement at a therapeutic dose, every medication change, and anything affecting an existing condition must be discussed with your physician first.

Three items here specifically require a physician or specialist before you act: the **Lp(a)/lipid** picture, the **iron depletion** (whose cause must be understood before high-dose iron), and the **chronic nasal obstruction** (ENT). Do not begin iron or any cardiovascular medication on the basis of this report alone.

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